

USER INFORMATION
READ CAREFULLY
CIPROXOL[®]
(Ciprofloxacin 0.2% w/v Intravenous Infusion BP)

Composition:
100 mL solution contain :-
Ciprofloxacin Lactate 254 mg equivalent to Ciprofloxacin 200 mg in Sodium Chloride BP 0.9% w/v.

Product Description:
CIPROXOL[®] is 0.2% w/v Intravenous Infusion, sterile and non-pyrogenic.

Pharmacology:
Pharmacodynamic:
Ciprofloxacin is a fluoroquinolone that differs from older quinolones (e.g. : nalidixic acid) in its potent activity against many Gram positive and Gram negative bacteria. Its mechanism of action involves inhibition of bacterial DNA- gyrase, an enzyme responsible for the unwinding of DNA for transcription and subsequent supercoiling of DNA for packaging into chromosomal subunits. It is highly active against Enterobacteriaceae, with MICs of often less than 0.1 mcg/mL. It is also very active against many strains of *Pseudomonas aeruginosa* and *Staphylococcus sp.* with a MIC₉₀ of 0.5 - 1 mcg/mL. It has poor activity against streptococci and anaerobes.

Pharmacokinetics:
200 mg IV dose infused over 30 mins results in a peak concentration of about 3.2 mcg/mL. V_d averages 2 L/kg. Less than 30% plasma protein bound. Ciprofloxacin attains very high concentrations in many body fluids and tissues, most notably urine, prostate and pulmonary mucosa. CSF concentrations are less than 1 mcg/ml: experience with the drug in the treatment of meningitis is very limited. 45 - 60% of a parenteral dose is recovered unchanged in urine: the remainder is excreted as four metabolites or eliminated in feces.

Indications:
Consideration should be given to official guidance on the appropriate use of antibacterial agents.

For CIPROXOL[®] solution for infusion:
Uncomplicated and complicated infections caused by Ciprofloxacin susceptible pathogens.
* Infections of the respiratory tract:
Ciprofloxacin can be regarded as an advisable treatment for pneumonia caused by *Klebsiella*, *Enterobacter*, *Proteus*, *E. coli*, *Pseudomonas*, *Haemophilus*, *Moraxella catarrhalis*, *Legionella* and *Staphylococcus*.
* Infections of the middle ear (otitis media), of the paranasal sinuses (sinusitis), especially if these are caused by Gram-negative organisms including *Pseudomonas aeruginosa* or by staphylococci.
Infections of the eyes
* Infections of the kidneys and/or the efferent urinary tract
Infections of the genital organs, including adnexitis, gonorrhea, prostatitis and excluding vaginal infections.
Infections of the abdominal cavity (e.g. infections of the gastrointestinal tract or of the biliary tract, peritonitis)
Infections of the skin and soft tissue
Infections of the bones and joints
* Sepsis
Infections or imminent risk of infection (prophylaxis) in patients whose immune system has been weakened (e.g. patients on immunosuppressants or have neutropenia)
* CIPROXOL[®] should be only used:
• When *Pseudomonas* is considered AND patient is allergic to antipseudomonal penicillins/cephalosporins; or
• For resistant organisms with no other alternative antibiotics available.

Recommended Dosage:
Dosage regimen
Unless otherwise prescribed, the following daily doses are recommended for;

Adults	
Table 1: Recommended daily doses of CIPROXOL [®] in adults	
Indication	Daily dose of Ciprofloxacin in mg for CIPROXOL [®] solution for infusion
Infections of the respiratory tract I (according to severity and organism)	2 x 400 mg to 3 x 400 mg
Urinary tract infections: - acute, uncomplicated	2 x 200 mg to 2 x 400 mg
- cystitis in women (before menopause)	N/A
- complicated	2 x 400 mg to 3 x 400 mg
Genital infections - uncomplicated gonorrhea (including extragenital sites of infection)	N/A
- adnexitis, prostatitis, epididymo-orchitis	2 x 400 mg to 3 x 400 mg
Diarrhea	2 x 400 mg
Other infections	2 x 400 mg
Particularly severe, life-threatening infections, i.e. - Recurrent infections in cystic fibrosis - Bone and joint infections - Septicemia - Peritonitis In particular when <i>Pseudomonas</i> , <i>Staphylococcus</i> or <i>Streptococcus</i> is present	3 x 400 mg
Inhalational anthrax (post exposure)	N/A
Prophylaxis of invasive infections due to <i>Neisseria meningitidis</i>	N/A

Missed dose
If a dose is missed, it should be taken as anytime but not later than 6 hours prior to the next scheduled dose. If less than 6 hours remain before the next dose, the missed dose should not be taken and treatment should be continued as prescribed with the next scheduled dose. Double doses should not be taken to compensate for a missed dose.

Additional information on special patient population

Children and adolescents
For recommended dosage regimen see Table 2.

Geriatric patients (> 65 years)
Elderly patients should receive a dose as low as possible depending on the severity of their illness and the creatinine clearance.

Patients with renal and hepatic impairment
Adults
Patients with renal impairment
Table 3: Recommended doses for patients with renal impairment

Creatinine Clearance [mL/min/ 1.73 m ²]	Serum Creatinine [mg/ 100 mL]	Total daily intravenous dose of Ciprofloxacin
30 to 60	1.4 to 1.9	maximum 800 mg
below 30	≥2.0	maximum 400 mg

• Patients with renal impairment on hemodialysis
- For patients with creatinine clearance between 30 and 60 mL/min/1.73 m² (moderate renal impairment) or serum creatinine concentration between 1.4 and 1.9 mg/ 100 mL, the maximum daily dose of Ciprofloxacin should be 800 mg for an intravenous regimen.
- For patients with creatinine clearance less than 30 mL/min/ 1.73 m² (severe renal impairment) or serum creatinine concentration equal or higher than 2.0 mg/ 100 mL, the maximum daily dose of Ciprofloxacin should be 400 mg for an intravenous regimen on dialysis days after dialysis.

• Patients with renal impairment on continuous ambulatory peritoneal dialysis (CAPD)
- Addition of CIPROXOL[®] solution for infusion to the dialysate (intraperitoneal): 50 mg Ciprofloxacin/ L dialysate administered 4 times a day every 6 hours.

• Patients with hepatic impairment
- In patients with impaired hepatic function no dose adjustment is required.

• Patients with renal and hepatic impairment
- For patients with creatinine clearance between 30 and 60 mL/min/ 1.73 m² (moderate renal impairment) or serum creatinine concentration between 1.4 and 1.9 mg/ 100 mL, the maximum daily dose of Ciprofloxacin should be 800 mg for an intravenous regimen.
- For patients with creatinine clearance less than 30 mL/min/ 1.73 m² (severe renal impairment) or serum creatinine concentration equal or higher than 2.0 mg/ 100 mL, the maximum daily oral dose of Ciprofloxacin should be 400 mg for an intravenous regimen.

Children and adolescents
Dosing in children with impaired renal and or hepatic function has not been studied.

Inhalational Anthrax (Post-exposure) in Adults and Children
For recommended dosage regimen see Table 1 (for adults) and Table 2 (for children and adolescents), respectively.

Route of Administration : Intravenous (IV)

CIPROXOL[®] solution for infusion should be administered by intravenous infusion over a period of 60 minutes. Slow infusion into a large vein will minimize patient discomfort and reduce the risk of venous irritation. The solution for infusion can be infused either directly or after mixing with other compatible solutions for infusion.
Unless compatibility with other solutions for infusion/ medicinal products has been confirmed, the solution for infusion must always be administered separately. The visual signs of incompatibility are e.g. precipitation, clouding and discoloration.
Incompatibility appears with all solutions for infusion/ medicinal products that are physically or chemically unstable at the pH of the solution (e.g. penicillins, heparin solutions), especially on combination with solutions adjusted to an alkaline pH (pH of CIPROXOL[®] solutions for infusion: 3.9 – 4.5). Only clear solutions are to be used.

Duration of treatment
The duration of treatment depends on the severity of the illness and on the clinical and bacteriological course. It is essential to continue therapy for at least 3 days after disappearance of the fever or of the clinical symptoms.

Mean duration of treatment
Adults
• 1 day for acute uncomplicated gonorrhea and cystitis
• up to 7 days for infections of the kidneys, urinary tract and abdominal cavity
• over the entire period of the neutropenic phase in patients with weakened body defenses
• a maximum of 2 months in osteomyelitis
• and 7 – 14 days in all other infections

In streptococcal infections, the treatment must last at least ten days because of the risk of late complications.
Infections caused by *Chlamydia spp.* should also be treated for a minimum of ten days.

Children and adolescents
• Cystic Fibrosis
For broncho-pulmonary infections of cystic fibrosis associated with *Pseudomonas aeruginosa* infection in paediatric patients (aged 5 – 17 years), the duration of treatment is 10 – 14 days.

• Complicated Urinary Tract Infections and Pyelonephritis
For complicated urinary tract infections or pyelonephritis due to *Escherichia coli*, the duration of treatment is 10 – 21 days.

Inhalational Anthrax (Post-exposure) in Adults and Children
60 days from the confirmation of Bacillus anthracis exposure

Contraindications:
Ciprofloxacin is contraindicated in patients who have shown hypersensitivity to ciprofloxacin or other quinolone anti-infectives.
Except in cases of exacerbation of cystic fibrosis associated with *P.aeruginosa* (in patients aged 5-17 years) and inhalation anthrax, Ciprofloxacin is contraindicated in children and growing adolescents unless the benefits of treatment are considered to outweigh the risks.
Concurrent administration of Ciprofloxacin and Tizanidine is contraindicated since an undesirable increase in serum Tizanidine concentrations associated with clinically relevant Tizanidine-induced side effects (hypotension, somnolence) can occur.

Warning and Precautions
The use of CIPROXOL[®] should be avoided in patients who have experienced serious adverse reactions in the past when using fluoroquinolones containing products. Treatment of these patients with CIPROXOL[®] should only be initiated in the absence of alternative treatment options and after careful benefit/ risk assessment.

Hypersensitivity
In the event of hypersensitivity, which in some instances can occur after the first administration, therapy should be discontinued.

Seizures
Ciprofloxacin should be used with caution in epileptics and patients with a history of CNS disorders and only if the benefits of treatment are considered to outweigh the risk of possible CNS side effects. CNS side effects have been reported after first administration of Ciprofloxacin in some patients. Treatment should be discontinued if the side effects, depression or psychoses lead to self-endangering behaviour.

Renal and Urinary Disorders
Crystalluria related to the use of Ciprofloxacin has been reported. Patients receiving Ciprofloxacin should be well hydrated and excessive alkalinity of the urine should be avoided.
Patients with a family history of or actual defects in glucose-6-phosphate dehydrogenase activity are prone to haemolytic reactions with quinolones, and so Ciprofloxacin should be used with caution in these patients.

Gastrointestinal System
There is a risk of pseudomembranous colitis with broad-spectrum antibiotics possibly leading to a fatal outcome. It is important to consider this in patients suffering from severe, persistent diarrhoea with Ciprofloxacin this effect has been reported rarely. If pseudomembranous colitis is suspected treatment with Ciprofloxacin should be stopped and appropriate treatment given (e.g., oral Vancomycin). Drugs that inhibit peristalsis must not be given.

Skin and Appendages
Ciprofloxacin has been shown to produce photosensitivity reactions. Patients taking Ciprofloxacin should avoid direct exposure to excessive sunlight or UV-light. Therapy should be discontinued if photosensitisation (i.e. sunburn-like skin reactions) occurs.

Hepatobiliary System
Laboratory tests may give abnormal findings if performed whilst patients are receiving Ciprofloxacin, e.g., increased alkaline phosphatase; increases in liver function tests, e.g., transaminases and cholestatic jaundice, especially in patients with previous liver damage.

Cardiac Disorders
In patients for whom sodium intake is of medical concern (e.g., patients with congestive heart failure, renal failure, nephrotic syndrome), the sodium content of CIPROXOL[®] Intravenous Infusion should be taken into account.

Exacerbation of Myasthenia Gravis

Fluoroquinolones have neuromuscular blocking activity and may exacerbate muscle weakness in person with myasthenia gravis. Post marketing serious adverse events, including deaths and requirement for ventilator support have been associated with fluoroquinolones use in persons with myasthenia gravis. Avoid fluoroquinolones in patients with known history of myasthenia gravis.

Aortic Aneurysm and Dissection

Epidemiologic studies report an increased risk of aortic aneurysm and dissection after intake of fluoroquinolones, particularly in the older population. Therefore, fluoroquinolones should only be used after careful benefit-risk assessment and after consideration of other therapeutic options in patients with positive family history of aneurysm disease, or in patients diagnosed with pre-existing aortic aneurysm and/or aortic dissection, or in presence of other risk factors or conditions predisposing for aortic aneurysm and dissection (e.g. Marfan syndrome, vascular Ehlers-Danlos syndrome, Takayasu arteritis, giant cell arteritis, Behcet's disease, hypertension, known atherosclerosis). In case of sudden abdominal, chest or back pain, patients should be advised to immediately consult a physician in an emergency department.

Prolonged, Disabling and Potentially Irreversible Serious Adverse Drug Reactions

Very rare cases of prolonged (continuing months or years), disabling and potentially irreversible serious adverse drug reactions affecting different, sometimes multiple body systems (musculoskeletal, nervous, psychiatric and senses) have been reported in patients receiving fluoroquinolones irrespective of their age and pre-existing risk factors. CIPROXOL® should be discontinued immediately at the first signs or symptoms of any serious adverse reaction and patients should be advised to contact their prescriber for advice.

Tendinitis and Tendon Rupture

Tendinitis and tendon rupture (especially but not limited to Achilles tendon), sometimes bilateral, may occur as early as within 48 hours of starting treatment with fluoroquinolones and have been reported to occur even up to several months after discontinuation of treatment. The risk of tendinitis and tendon rupture is increased in older patients (above 60 years of age), with renal impairment, patients with solid organ transplants, and those treated concurrently with corticosteroids*. Therefore, concomitant use of corticosteroids should be avoided. At the first sign of tendinitis (e.g., painful swelling, inflammation) the treatment with CIPROXOL® should be discontinued and alternative treatment should be considered. The affected limb(s) should be appropriately treated (e.g., immobilisation). Corticosteroids should not be used if signs of tendinopathy occur.

Peripheral Neuropathy

Cases of sensory or sensorimotor polyneuropathy resulting in paraesthesia, hypaesthesia, dysesthesia, or weakness have been reported in patients receiving quinolones and fluoroquinolones. Patients under treatment with CIPROXOL® should be advised to inform their doctor and pharmacist prior to continuing treatment if symptoms of neuropathy such as pain, burning, tingling, numbness, or weakness develop in order to prevent the development of potentially irreversible condition (see section Adverse Effects/Undesirable Effects).

Psychiatric reactions

Psychiatric reactions may occur even after the first administration of fluoroquinolones, including CIPROXOL®. In rare cases, depression or psychotic reactions can progress to suicidal ideations/thoughts and self-injurious behaviour, such as attempted or completed suicide (see section 'Undesirable effects'). In the event that the patient develops these reactions, CIPROXOL® should be discontinued and appropriate measures instituted. Caution is recommended if CIPROXOL® is to be used in psychotic patients or in patients with a history of psychiatric disease.

Interactions with Other Medicaments

Increased plasma levels of Theophylline have been observed following concurrent administration with Ciprofloxacin. It is recommended that the dose of Theophylline should be reduced and plasma levels of Theophylline monitored. The reaction between Theophylline and Ciprofloxacin is potentially life-threatening. Therefore, where monitoring of plasma levels is not possible, the use of Ciprofloxacin should be avoided in patients receiving Theophylline. Particular caution is advised in those patients with convulsive disorders.

Ciprofloxacin inhibits CYP1A2 and thus may cause increased serum concentration of concomitantly administered substances metabolised by this enzyme (e.g. Theophylline, Clozapine, Tacrine, Ropinirol, Tizanidine). Therefore, patients taking these substances concomitantly with Ciprofloxacin should be monitored closely for clinical signs of overdose, and determination of serum concentrations, especially of Theophylline, may be necessary.

In a crossover study, 10 healthy subjects were given Ciprofloxacin 500 mg or placebo twice daily for three days, at the end of which a single dose of Tizanidine 4 mg was given. There was an increase in Tizanidine serum concentrations (Cmax increase: 7-fold, range: 4 to 21-fold; AUC increase: 10-fold, range: 6 to 24 fold) when given concomitantly with Ciprofloxacin compared to placebo. Associated with the increased serum concentrations was a potentiated hypotensive and sedative effect. Tizanidine must not be administered together with Ciprofloxacin.

Phenytoin levels may be altered when CIPROXOL® is used concomitantly.

Prolongation of bleeding time has been reported during concomitant administration of Ciprofloxacin and oral anti-coagulants.

Animal data have shown that high doses of quinolones in combination with some non-steroidal anti-inflammatory drugs, (e.g fenbufen, but not acetylsalicylic acid) can lead to convulsions.

Transient increases in serum creatinine have been seen following concomitant administration of Ciprofloxacin and Cyclosporin. Therefore, monitoring of serum creatinine levels is advisable.

The simultaneous administration of quinolones and glibenclamide can on occasion potentiate the effect of glibenclamide resulting in hypoglycaemia.

Renal tubular transport of methotrexate may be inhibited by concomitant administration of Ciprofloxacin potentially leading to increased plasma levels of methotrexate. This may increase the risk of methotrexate associated toxic reactions. Therefore, patients receiving methotrexate therapy should be carefully monitored when concomitant Ciprofloxacin therapy is indicated.

Concomitant use with probenecid reduces the renal clearance of Ciprofloxacin, resulting in increased quinolone plasma levels.

Pregnancy and Lactation:

The use of Ciprofloxacin should be avoided in pregnancy and lactation. No safety data is available for use in pregnancy and lactation.

On the basis of animal studies, it is not entirely improbable that the drug could cause damage to articular cartilage in the immature organism. Animal studies have not shown any evidence of teratogenic effects (malformations).

Adverse Effects / Undesirable Effect:

The most frequently reported adverse reactions are : nausea, diarrhoea and rash.

Adverse reactions are listed by frequency :	Common	: >1/100,	<1/10
	Uncommon	: >1/1000,	<1/100
	Rare	: >1/10000,	<1/1000
	Very Rare	: <1/10000	

The following adverse reactions have been observed:

Adverse Reactions	Common	Uncommon	Rare	Very Rare
Infections and Infestations	-	Moniliasis	Moniliasis (oral), vaginal moniliasis	Moniliasis (gastrointestinal)
Blood and The Lymphatic System Disorders	-	Eosinophilia, Leukopenia	Anaemia, Leukopenia (granulocytopenia), Leucocytosis, Thrombocytopenia, Thrombocythaemia (thrombocytosis)	Hemolytic anaemia, Agranulocytosis, Pancytopenia (life-threatening), Bone marrow depression (life-threatening)
Immune System Disorders	-	-	Allergic	Shock (anaphylactic/anaphylactoid reactions progressing in very rare cases to life-threatening shock), serum sickness like reaction, angioedema, pruritic rash.
Metabolism and Nutrition Disorders	-	Anorexia	Hyperglycaemia	-
*Psychiatric and Nervous System Disorders	-	Headache, dizziness, insomnia, agitation, confusion, taste perversion (usually reversible upon discontinuation of treatment)	Hallucination, paresthesia (peripheral paralgesia), abnormal dreams (nightmares), depression, tremor, convulsion, hypesthesia, somnolence, exacerbation of symptoms of myasthenia gravis.	Grand mal convulsion, abnormal gait, psychosis (which may progress to self-endangering behaviour), intracranial hypertension, ataxia, hyperesthesia, hypertonia, parosmia (impaired smell), anosmia (usually reversible on discontinuation), migraine, anxiety, taste loss (impaired taste).
*Eye Disorders	-	-	-	Abnormal vision (visual disturbances), diplopia, chromatopsia.
*Ear and Labyrinth Disorders	-	-	-	Tinnitus, transitory deafness (especially at high frequencies)
Cardiac and Vascular Disorders	-	-	Tachycardia, syncope (fainting), vasodilation (hot flushes), hypotension	Vasculitis (petechiae, haemorrhagic bullae, papules, crust formation)
Gastrointestinal Disorders	Nausea, diarrhoea	Vomiting, dyspepsia, flatulence, abdominal pain.	Dysphagia, pseudomembranous colitis, pancreatitis.	Life-threatening pseudomembranous colitis with possible fatal outcome.
Hepato-biliary Disorders	-	Bilirubinaemia	Jaundice, cholestatic jaundice, liver necrosis (rarely progressing to life-threatening hepatic failure)	Hepatitis
Skin and Subcutaneous Tissue Disorders. Reaction, anaphylactoid (anaphylactic) reaction, dyspnoea, larynx oedema	Rash	Pruritis, maculopapular rash, urticaria	Photosensitivity reaction, erythema multiforme (minor), erythema nodosum	Stevens-Johnson-Syndrome, epidermal necrolysis (LyeII-Syndrome), fixed drug reaction, petechiae
*Musculoskeletal, Connective Tissue and Bone Disorders	-	Arthralgia	Myalgia, joint disorder (joint swelling), pain in extremities, back pain, myasthenia	Twitching, tendinitis (predominantly achillo tendinitis including tenosynovitis), partial or complete tendon rapture (predominantly achilles tendon)
Renal and Urinary Disorders	-	-	Acute kidney failure, abnormal kidney function, haematuria, crystalluria, interstitial nephritis	-
*General Disorders and Administration Site Conditions	-	Asthesia (general feeling of weakness, tiredness), injection site reaction (e.g oedema/ hypersensitivity/ inflammation/ pain), (thrombo)-phlebitis (at infusion site)	Pain, chest pain, sweating, drug fever, oedema (peripheral, vascular, face)	-
Investigations	-	SGOT increased, SGPT increased, abnormal liver function test, alkaline phosphatase increased, increases in creatinine, increases in BUN (urea)	Altered prothrombin values	Amylase increased, lipase increased, myasthenia gravis
Psychiatric disorders	-	-	Depression (potentially culminating in suicidal ideations/ thoughts or suicide attempts and completed suicide)	Psychotic reactions (potentially culminating in suicidal ideations/ thoughts or suicide attempts and completed suicide)

**Very rare cases of prolonged (up to months or years), disabling and potentially irreversible serious drug reactions affecting several, sometimes multiple, system organ classes and senses (including reactions such as tendinitis, tendon rupture, arthralgia, pain in extremities, gait disturbance, neuropathies associated with paraesthesia, depression, fatigue, memory impairment, sleep disorders, and impairment of hearing, vision, taste and smell) have been reported in association with the use of fluoroquinolones in some cases irrespective of pre-existing risk factors.*

Exacerbation of myasthenia gravis

Post Marketing Experience

Overdose and Treatment:

Symptoms:

In the cases of overdosage, reversible nephrotoxicity has been occasionally reported. Therefore, besides normal emergency measures, it is suggested that renal function be monitored and antacids containing Magnesium or Calcium, which reduce Ciprofloxacin absorpton, be administered. Only a small fraction (10%) of Ciprofloxacin is removed from the body by hemodialysis or peritoneal dialysis.

User Instruction & Pharmaceutical Precaution

Precipitation may occur at cool storage temperature, which will redissolve at room temperature. It is not recommended to store the product in refrigerators.

Incompatibilities:

CIPROXOL® is compatible with physiological saline, Ringer solution and Ringer Lactate Solution, 5% and 10% Glucose Solutions, and 5% Glucose solution with 0.45% Sodium Chloride. When CIPROXOL® are mixed with compatible solutions for infusion, for microbiological reasons and light sensitivity these solutions should be administered shortly after admixture. Unless compatibility with other solutions for infusion/ medicinal products has been confirmed, the solution for infusion must always be administered separately. The visual signs of incompatibility are e.g. precipitation, clouding, and discoloration. Incompatibility appears with all solutions for infusion/medicinal products that are physically or chemically unstable at the pH of the solution (e.g. penicillins, heparin solutions), especially on combination with solutions adjusted to an alkaline pH (pH of the CIPROXOL®: 3.9 – 4.5).

Storage Conditions:

Do not store above 30°C. Protect from light. Do not refrigerate.

Shelf Life:

2 years from manufacturing date in the proper storage condition. Do not use after expiry.

Dosage Forms and Packaging Available:

50 mL X 20 LDPE plastic bottle per Carton.
100 mL X 20 LDPE plastic bottle per Carton.

Manufacturer / Product Registration Holder:

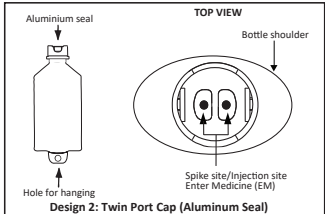
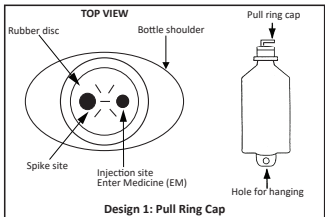
 **AIN MEDICARE SDN.BHD.**
Lot 4933, 4934 & PT2464, Jalan 6/44, Kaw. Perindustrian Pengkalan Chepa 2, 16100 Kota Bharu, Kelantan, MALAYSIA.

HANDLING INSTRUCTION

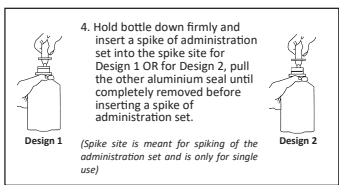
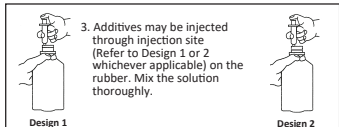
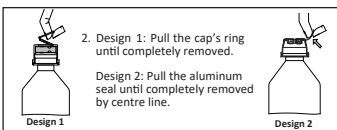
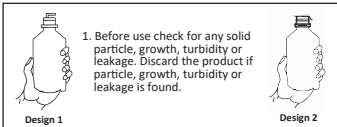
AIN MEDICARE INTRAVENOUS INFUSION FLUID IN LOW DENSITY POLYETHYLENE (LDPE) BOTTLE

The bottle is made from injectable grade polyethylene.

PARTS OF THE PLASTIC BOTTLE



Top view after pull ring cap is removed. Please refer whichever applicable design.
INSTRUCTION FOR THE USAGE OF IV ADMINISTRATION SET & ADDITION OF DRUG



Date of Revision : 01.11.2024